

DISCHARGE MEDICATION RECONCILIATION & MEDICATION-SAFETY REVIEW

Mercy Vale Regional Medical Center — Hospital Medicine / Inpatient Pharmacy

Patient: Korvin Merrow | **MRN:** KM-6427819 | **DOB:** 02/18/1964 (62 y, M) | **FIN:** KM-2026-051877 **Unit/Room:** 5 West Medical / 5W-318 | **Allergy:** Lisinopril (cough — ACE-I intolerance) **Anticipated discharge:** 05/24/2026 | **Note date:** 05/24/2026 | **Status:** For co-signature

Basis: Verified 19-item home list (Marlott PharmD, 05/18) reconciled against the MAR (HD1–HD6); Cardiology (Caldrane), Nephrology (Solthar), Endocrinology (Veylorn), Rheumatology (Halvek) records; pharmacy fills; trend summary; PT/OT, nursing, Case Management, family-conference notes. AKI-on-CKD resolved to baseline by HD6 (Cr 2.62→1.80, K 5.1→4.4, SBP 96→114–130, WBC 15.6→9.4, afebrile).

CONTINUED — home dose, documented outpatient indication

Aspirin 81 mg daily and atorvastatin 40 mg nightly (CAD secondary prevention, post mid-LAD DES; DAPT completed remotely, so aspirin monotherapy is correct); pantoprazole 40 mg daily (GERD); calcium carbonate/vitamin D BID and alendronate 70 mg weekly-Sunday (bone health; resume home schedule); ferrous sulfate 325 mg every other day (anemia of CKD); and the PRN set — nitroglycerin 0.4 mg SL (angina rescue), acetaminophen 650 mg q6h (**avoid NSAIDs — renal**), PEG 3350 and senna (bowel). All continued/tolerated inpatient.

DOSE-MODIFIED / CHANGED

- **Insulin glargine 18 units SC nightly** — titrated 12→18 units inpatient; back to home dose by HD6. Resume 18 units nightly; PCP/Endocrinology to reconcile the glycemic regimen (A1c 7.6–8.2%). No new agent started in-hospital.
- **Gabapentin 300 mg nightly** — reduced to 100 mg for confusion/fall risk, then back to 300 mg HD5–6 with improved mentation. Resume 300 mg nightly; monitor sedation/falls.

HELD WITH RESTART PARAMETERS — staged, NOT simultaneous

Both consultants explicitly advise staged **outpatient** restart, not simultaneous reintroduction at discharge. - **Sacubitril/valsartan 24/26 mg BID** — held HD1–6; first candidate for outpatient reintroduction, gated on Cr at baseline, K <5.0, adequate SBP. Restart by Cardiology/Nephrology at follow-up, not at discharge. - **Spironolactone 25 mg daily** — held HD1–6; later in sequence, gated on K and eGFR. - **Empagliflozin 10 mg daily** — held HD1–6; restart once intake/renal function stable (Endocrinology defers timing to renal/cardiac teams). - **Furosemide 40 mg daily** — held HD1–6 (now euvolemic); restart only if volume reaccumulates, gated on home weight vs 97 kg dry weight. - **Metformin ER 500 mg BID** — held HD1–6 (AKI); restart once renal function confirmed stable, via PCP/Endocrinology. - **Carvedilol 12.5 mg BID** — *reintroduced and tolerated inpatient* (half-dose HD3, full BID HD4–

6, SBP 116–124); Cardiology cautioned against beta-blocker lapse. **Continue 12.5 mg BID; hold a dose if SBP <100 or symptomatic.**

PREDNISONE (cross-source reconciliation)

No verified numeric home dose. Fills trended 10 mg (Nov–Jan) → 5 mg (Feb–May), with “½ tablet daily” 03/22 and most recent dispense **5 mg, taper-style directions, 05/06/2026** (Halvek). Rheumatology documents a cautious symptom-guided slow taper — avoid abrupt discontinuation — without a number; Endocrinology advises continuing non-abrupt coverage and **not inventing a new long-term dose inpatient. Disposition:** discharge on **prednisone 5 mg PO daily as an explicitly provisional bridge** (matching inpatient interim coverage); **do not stop abruptly**; definitive taper set at Rheumatology follow-up.

INPATIENT-SPECIFIC — do NOT carry forward

Ceftriaxone 1 g IV (HD1–3), insulin lispro correctional sliding scale, and IV fluids had inpatient indications only; no outpatient prescription. **Antibiotic disposition:** empiric therapy for urinary-source infection with clinical resolution (afebrile, WBC 15.6–9.4); culture remained preliminary/unspecified. After ceftriaxone HD1–3 and oral step-down cefpodoxime 200 mg PO BID HD4–6, **continue cefpodoxime 200 mg PO BID one additional day to complete a 7-day course, then STOP (05/25/2026)** — no further outpatient antibiotic.

POST-DISCHARGE RENAL/CARDIAC MONITORING

BMP (creatinine/eGFR + potassium) 48–72 h post-discharge at Mercy Vale Outpatient Laboratory — Harbor Crest, (412) 555-0455; results to PCP (Quenor) and Nephrology (Solthar). Daily home weight vs 97 kg dry weight — call for gain >2 kg over 2–3 days. Call for SBP <100 or symptomatic lightheadedness, or sustained SBP >150. Confirm transportation and lab logistics (unconfirmed per Case Management).

HOME MEDICATION-MANAGEMENT SAFETY PLAN

OT found reproducible BID-timing and PRN/scheduled errors; nursing documented he could not reliably confirm whether doses were taken — a deviation from baseline (mild forgetfulness with family oversight) requiring support. - **Pre-filled weekly pill organizer** filled by caregiver/pharmacy — **not** Korvin’s sole responsibility early on. - **Confirm home-health nursing** (Keystone HomeCare) for early medication review; confirm first-fill pickup (Harbor Crest Community Pharmacy). - **Daily caregiver check at medication times for 5–7 days**, verifying the dose was taken; name the caregiver for **weekday-morning doses** (Lenora unavailable weekday AM). - Rolling walker (not cane), bedside commode, grab bars, remove loose rugs; supervised mobility 5–7 days (Morse 65).

TEACH-BACK (patient + Mara Merrow, HCP/caregiver; Lenora by phone; confirm at session and at 24 h)

1. **Changed — carvedilol:** 12.5 mg twice daily; skip and call if dizzy or home SBP <100.
2. **Held/not-yet-restarted:** sacubitril/valsartan, spironolactone, empagliflozin, furosemide, metformin are **intentionally held** and restarted one at a time after lab checks — do **not** resume from old bottles.

3. **Prednisone:** 5 mg daily for now, **do not stop suddenly**; Rheumatology sets the taper. If fever, vomiting, or dehydration, call before changing it.
4. **Glycemic:** glargine 18 units nightly; no home sliding-scale insulin; PCP/Endocrinology to review.
5. **Warning signs → call:** weight gain, worsening swelling/breathlessness, chest pain, recurrent weakness/lightheadedness/near-fall, new confusion, poor intake, or fever — PCP or ED (412) 555-0150.

FOLLOW-UP

PCP (Quenor) ≤7 days; Nephrology (Solthar) 1–2 wk; Cardiology (Caldrane) 2 wk (GDMT restart titration); Endocrinology (Veylorn) 2–4 wk; Rheumatology (Halvek) 2–4 wk (prednisone taper).

Reviewing Pharmacist: _____ Ezekiel Marlott, PharmD, BCPS —
Inpatient Pharmacy Date/Time: _____

Covering Hospitalist: _____ Hospital Medicine (for E. Vossmere, MD)
Date/Time: _____

*Co-signature required before release with discharge paperwork. Pharmacy (412) 555-0170 /
Hospital Medicine pager (412) 555-0301.*